

CoverWise AI

Coverage Insight and Risk Intelligence Report
Generated 02 Oct 2026, 09:44

Important: CoverWise AI is a policy-understanding and decision-support assistant. It does NOT provide financial, legal, medical or insurance advice and does not recommend buying, switching or cancelling any policy. AI extraction can be incomplete or wrong. Always verify every point with the official policy wording, the insurer, or a licensed insurance advisor before making a decision.

User profile used for this analysis

Field	Value
Age	35
Family Members	Self, Spouse
Has Ped	No
Planning Child	No
Adding Child	No

Current policy

Coverage Clarity Score: 37/100 – High Risk / Needs Advisor Review

Scoring method: ML (Logistic Regression) + rule-based explanation | model confidence 100% | transparent rule score 35/100 (High Risk)

Easy-to-read summary

The policy is the IFFCO-Tokio “Arogya Sanjeevani” individual health plan, renewable yearly for a family (self, spouse, parents, parents-in-law, and dependent children 3 months-25 years).

You can choose a sum insured from INR 50,000 up to INR 1,000,000; the premium amount is not stated in the document (verify).

It covers inpatient hospitalization, pre-hospitalisation (up to 30 days) and post-hospitalisation (up to 60 days) expenses, as well as day-care procedures and ambulance charges up to INR 2,000 per stay.

A 5 % co-payment is applied to every admissible claim amount, meaning you pay that share and the insurer pays the rest [S4].

Room-rent is reimbursed up to 2 % of the sum insured, capped at INR 5,000 per day; ICU charges up to 5 % of the sum insured, capped at INR 10,000 per day.

There is a 30-day waiting period for any illness from policy start, except for accidents; this does not apply if you have continuous coverage for more than 12 months [S1].

Specific diseases and surgeries have a longer waiting period of 24-36 months of continuous coverage, again waived for accidents [S2].

No-claim bonus increases the sum insured by 5 % for each claim-free year, up to a maximum of 50 % of the original sum insured.

Claims can be made cash-less at network hospitals or by reimbursement; you must inform the insurer within 24 hours of emergency admission or before discharge, and at least 48 hours before a planned admission.

The insurer aims to settle cash-less or reimbursed claims within 30 days of receiving all documents, or up to 45 days if an investigation is needed.

The policy can be migrated to another IFFCO-Tokio product with at least 30 days' notice before renewal, and continuous prior coverage can reduce waiting periods [S6].

Certain expenses are excluded, such as diagnostic admissions not related to treatment, custodial care, and many pre-existing disease costs during the waiting periods; a full list of permanent and temporary exclusions is in the policy (verify details).

Top reasons behind the score

Factor	Value	Points
each risk flag	7	-49.0
per vague clause	6	-12.0
per month of PED waiting	36	-9.0
no-claim / cumulative bonus available	1	+5.0
per % co-payment	5.0	-3.8
child cover available	1	+0.5

Coverage and restriction report

Clause	Extracted value	Source
Insurer	IFFCO-Tokio General Insurance Company Limited	p.1
Policy / plan	Arogya Sanjeevani Policy	p.1
UIN	IFFHLIP20161V011920	-
Policy type	Individual Health	p.19
Sum insured	Not clearly found	-
Annual premium	Not clearly found	-
In-patient hospitalisation	Yes	p.6
Pre-hospitalisation days	30	p.7
Post-hospitalisation days	60	p.19
Day care	Yes	p.6
Ambulance	Road ambulance up to Rs2000 per hospitalization	p.6
AYUSH	Yes	p.6
Domiciliary	No	p.11
Restoration benefit	Not clearly found	-
No-claim bonus	Yes	p.19
Maternity	No	p.11
Newborn	Not clearly found	-
Dependent child	Yes	p.19
Initial waiting (days)	30	p.8
PED waiting (months)	36	p.8
Specified disease waiting (months)	36	p.8
Co-payment %	5.0	p.13
Co-payment conditions	Applicable to all admissible claim amounts	p.13
Room rent limit	up to 2% of sum insured subject to maximum Rs5000 per day	p.6
ICU limit	up to 5% of sum insured subject to maximum Rs10000 per day	p.6
Disease sub-limits	Cataract: The Company shall indemnify medical expenses incurred for treatment of Cataract, subject to a limit of 25% of Sum Insure; Modern Treatments: 4.6. The following procedures will be covered (wherever medically ind	p.7
Exclusions (count)	28	-
Cashless	Yes	p.12
Reimbursement	Yes	p.12
Claim intimation	Within 24 hours of emergency hospitalization or before discharge, whichever is earlier; at least 48 hours prior to planned admission	p.12
Grace period (days)	30	p.16
Portability described	Yes	p.16

Risk flags and caution points

Severity	Flag	Why it matters	Source
High	Long pre-existing disease waiting period (>= 36 months)	Claims for conditions you already have may not be paid for several years.	p.8
High	Room rent is capped	Choosing a costlier room can reduce the whole claim proportionately, not just the room charge.	p.6
Medium	ICU charges are capped	Long ICU stays can exceed the cap and the excess is paid by you.	p.6
Medium	Large number of exclusions (>= 15)	More situations where claims are simply not payable.	-
Medium	Several vague / discretionary clauses (>= 3)	Vague wording leaves room for disputes at claim time.	-
Medium	No restoration / recharge of sum insured	If the sum insured is used up, there is no automatic top-up in the same year.	-
Medium	Long specified-disease waiting period (>= 24 months)	Listed illnesses/surgeries are not covered for the first 2+ years.	p.8
Verify	Annual premium not clearly found in the document	A typical value was assumed for scoring - confirm the real value.	-
Verify	Restoration benefit not clearly found in the document	A typical value was assumed for scoring - confirm the real value.	-

Unclear clauses to verify

- p.27: "Annexure - C : List of Day Care Procedures ITGI "List of Day Care Procedures" Note: The list of such treatments is dynamic and hence may change from t"
- p.7: "The Company shall indemnify medical expenses incurred for inpatient care treatment under Ayurveda, Yoga and Naturopathy, Unani, Siddha and Homeopathy "
- p.19: "Name Arogya Sanjeevani Policy, IFFCO-Tokio General Insurance Company Limited Product Type Individual/ Floater Category of Cover Indemnity Sum insured "
- p.12: "1.2 Procedure for reimbursement of claims: For reimbursement of claims the insured person may submit the necessary documents to TPA(if applicable)/Com"
- p.1: "If during the policy period one or more Insured Person (s) is required to be hospitalized for treatment of an Illness or Injury at a Hospital/ Day Car"
- p.8: waiting period has options '24/36 months' - depends on plan/schedule or condition; longest used for scoring
- Specific Waiting Period: until expiry of 24/36 months of continuous coverage
- ii. 48 Months waiting period

Proposed policy

Coverage Clarity Score: 69/100 – Moderate Coverage

Scoring method: ML (Logistic Regression) + rule-based explanation | model confidence 86% | transparent rule score 70/100 (Moderate Coverage)

Easy-to-read summary

This is a family-floater health plan called “my: Optima Secure” offered by HDFC ERGO, with sum-insured choices of Rs 10 K, Rs 25 K, Rs 50 K, Rs 1 Lakh or Rs 2 Lakhs.

The base cover pays for inpatient hospitalization, including pre-hospitalisation (up to 60 days) and post-hospitalisation (up to 180 days), and also covers day-care procedures.

Ambulance services are covered, though the exact monetary limit is not stated in the document.

There is no co-payment required; the insurer pays the full eligible amount.

Room-rent and ICU charges are reimbursed at actual cost unless the policy schedule specifies a different limit[S3].

A 30-day waiting period applies to all non-accident illnesses from the policy start date, but it is waived after 12 months of continuous coverage [S2].

Specified diseases and procedures have a 24-month waiting period, reduced if the person has prior continuous coverage under portability rules [S4].

Pre-existing disease (PED) waiting period can be modified to 1 year or 2 years depending on the sum-insured option [S5].

The plan includes a “Restoration Benefit” that adds an extra amount to the sum insured for eligible claims, but the exact amount is not detailed in the excerpts (verify).

No-claim bonus is available, with the total liability for the year remaining the sum insured, but the bonus calculation details are not fully described (verify).

Claims can be made cash-less at network hospitals or by reimbursement; you must inform the insurer within 24 hours of emergency admission or at least 48 hours before a planned admission, and the insurer aims to settle within 15 days of receiving the intimation.

The policy renews automatically (subject to no fraud or mis-disclosure) with a 30-day grace period, no loading on renewal, and no fresh underwriting unless the sum insured is increased.

Top reasons behind the score

Factor	Value	Points
each risk flag	4	-28.0
per vague clause	6	-12.0
restoration benefit available	1	+10.0
per month of PED waiting	36	-9.0
no-claim / cumulative bonus available	1	+5.0
child cover available	1	+0.5

Coverage and restriction report

Clause	Extracted value	Source
Insurer	HDFC ERGO General Insurance Company Limited	p.1
Policy / plan	my: Optima Secure	-
UIN	HDFHLIP25041V062425	-
Policy type	Family Floater	-
Sum insured	Not clearly found	-
Annual premium	Not clearly found	-
In-patient hospitalisation	Yes	p.11
Pre-hospitalisation days	60	p.12

Clause	Extracted value	Source
Post-hospitalisation days	180	p.12
Day care	Yes	p.11
Ambulance	covered (limit not specified)	p.11
AYUSH	Yes	p.12
Domiciliary	Yes	p.12
Restoration benefit	Yes	p.17
No-claim bonus	Yes	p.2
Maternity	No	p.32
Newborn	Not clearly found	-
Dependent child	Yes	p.14
Initial waiting (days)	30	p.30
PED waiting (months)	36	p.28
Specified disease waiting (months)	24	p.28
Co-payment %	0.0	p.41
Co-payment conditions	No co-payment shall apply if Insured Person from Tier 2 avails a treatment in Tier 1. Instalment Premium payment through Auto Debit/ECS Facility a. If premium payment is opted for by instalments throu	-
Room rent limit	At Actuals unless otherwise specified in the Policy Schedule	-
ICU limit	At Actuals unless otherwise specified in the Policy Schedule	-
Disease sub-limits	Not clearly found	-
Exclusions (count)	28	-
Cashless	Yes	p.41
Reimbursement	Yes	p.43
Claim intimation	Within 24 hours of emergency hospitalization or before discharge, whichever is earlier; at least 48 hours prior for planned admission	p.41
Grace period (days)	30	p.37
Portability described	Yes	-

Risk flags and caution points

Severity	Flag	Why it matters	Source
High	Long pre-existing disease waiting period (>= 36 months)	Claims for conditions you already have may not be paid for several years.	p.28
Medium	Large number of exclusions (>= 15)	More situations where claims are simply not payable.	-
Medium	Several vague / discretionary clauses (>= 3)	Vague wording leaves room for disputes at claim time.	-
Medium	Long specified-disease waiting period (>= 24 months)	Listed illnesses/surgeries are not covered for the first 2+ years.	p.28
Verify	Annual premium not clearly found in the document	A typical value was assumed for scoring - confirm the real value.	-
Verify	Extraction cross-check	room_rent_limit_present: LLM read 'True', rule extractor found 'False' (p.11) - kept the rule value (base wording); verify in the document.	-
Verify	Extraction cross-check	icu_limit_present: LLM read 'True', rule extractor found 'False' (p.11) - kept the rule value (base wording); verify in the document.	-
Verify	Extraction cross-check	policy_term_years=5.0 is outside the plausible range [1, 3] - ignored.	-

Unclear clauses to verify

- p.43: "b. The Company reserves the right to modify, add or restrict any Network Provider for Cashless facility at its sole discretion."
- p.44: "If requested by the Company, at the Company's cost, the Insured Person must submit to medical examination by Medical Practitioner appointed by the Com"

- p.2: "Operating Clause If during the Policy Period the Insured Person is required to be Hospitalized for treatment of an Illness or Injury at a Hospital/ Da"
- p.36: "Premium Payment in Instalments If the Insured Person has opted for payment of Premium on an instalment basis i.e. Yearly, Half Yearly, Quarterly or Mo"
- p.17: "A single claim in the Policy Year shall never exceed the cumulative addition of a. Base Sum Insured, b. Cumulative Bonus (if applicable and remaining "
- p.44: "k. History of alcohol consumption or any intoxication certified by first treating doctor in case of Accident cases, l. Copy of Death Summary and copy "
- p.11: "The Hospitalization must be for Medically Necessary Treatment, and prescribed in writing by Medical Practitioner."
- p.11: room rent limit depends on the plan / Policy Schedule - check your schedule

Current policy vs proposed policy

Aspect	Current	Proposed	Change
Sum insured (Rs)	500,000	1,000,000	Improved
Annual premium (Rs)	Not provided	Not provided	Unclear
Pre-existing disease waiting (months)	36	36	Same
Specified disease waiting (months)	36	24	Improved
Initial waiting period (days)	30	30	Same
Co-payment (%)	5	0	Improved
Room rent capped	Yes	No	Improved
ICU charges capped	Yes	No	Improved
Disease-wise sub-limits (count)	2	0	Improved
Exclusions (count)	28	28	Same
Restoration / recharge benefit	Not found	Yes	Unclear
No-claim / cumulative bonus	Yes	Yes	Same
Pre-hospitalisation (days)	30	60	Improved
Post-hospitalisation (days)	60	180	Improved
Day care procedures	Yes	Yes	Same
AYUSH treatment	Yes	Yes	Same
Maternity cover	No	No	Same
Newborn cover	Not found	Not found	Unclear
Dependent child cover	Yes	Yes	Same
Claim process clarity (0-100)	100	100	Same
Coverage Clarity Score	37	69	Improved

This policy appears stronger in these areas

Sum insured (Rs), Specified disease waiting (months), Co-payment (%), Room rent capped, ICU charges capped, Disease-wise sub-limits (count), Pre-hospitalisation (days), Post-hospitalisation (days), Coverage Clarity Score

This policy needs caution in these areas

None detected

Unclear - must be verified

Annual premium (Rs), Restoration / recharge benefit, Newborn cover

Premium vs coverage

Premiums were not found in the documents - enter both premiums in the sidebar to compare cost against the 9 improved and 0 weaker aspect(s).

Switching risks

- Waiting periods (pre-existing, specified diseases, maternity) normally restart with a new policy. Under IRDAI portability rules you can ask for credit of the waiting period already served, usually only up to the previous sum insured - get this in writing before switching.
- Any accumulated no-claim / cumulative bonus on the current policy may not fully carry over.
- Do not let the current policy lapse until the new one is issued and the portability request is accepted (keep a gap-free continuity).
- For the increased part of the sum insured, waiting periods usually apply afresh.

AI explanation of the comparison

Where the proposed policy appears stronger

- Higher sum insured – the base cover is doubled from Rs 5 lakhs to Rs 10 lakhs. [Proposed Extraction]
- Shorter specific-disease waiting period – reduced from 36 months to 24 months. [Proposed Extraction]
- No co-payment – the proposed plan has 0 % co-payment versus 5 % in the current plan. [Proposed Extraction]
- Room-rent and ICU limits removed – no caps on room-rent or ICU charges, whereas the current plan caps them (room-rent up to 2 % of SI, max Rs 5,000/day; ICU up to 5 % of SI, max Rs 10,000/day). [Current Extraction] / [Proposed Extraction]
- No disease-wise sub-limits – the proposed plan has none, while the current plan applies sub-limits (e.g., cataract 25 % of SI). [Current Extraction]
- Longer pre-hospitalisation and post-hospitalisation coverage – 60 days pre-hospitalisation (vs 30) and 180 days post-hospitalisation (vs 60). [Proposed Extraction]
- Restoration / recharge benefit is offered – the proposed policy includes a restoration benefit; the current policy does not mention one. [Proposed Extraction]
- Higher coverage-clarity score – 69 versus 37 for the current plan, indicating clearer wording of benefits. [Comparison Table]

Where the proposed policy needs caution / appears weaker

- Premium amount not disclosed – the table shows “Unclear” for both policies, so you cannot compare cost. [Comparison Table]
- Optional add-ons not detailed – many benefits (e.g., Global Health Cover, aggregate deductible waiver) are mentioned only as optional, so actual coverage may vary. [Proposed Extraction]
- Ambiguities in policy wording – the proposed document notes several unclear clauses (e.g., network provider changes, exact room-rent limits, restoration benefit details). [Proposed Extraction]
- Exclusions count unchanged – both policies list 28 exclusions; the same number of exclusions does not guarantee the same items, and some exclusions are extensive. [Comparison Table]

Unclear points that must be verified

- Exact annual premium for both plans – needed to assess cost-vs-benefit. [Comparison Table]
- Details of the restoration / recharge benefit – how much extra sum insured is provided and under what conditions. [Proposed Extraction]
- Whether the “no-claim / cumulative bonus” works identically – wording differs; confirm the maximum bonus and eligibility. [Current Extraction] / [Proposed Extraction]
- Scope of “room-rent at actuals” – confirm any ceiling or documentation required. [Proposed Extraction]
- Exact limits for ambulance cover – the proposed policy mentions coverage but no monetary cap. [Proposed Extraction]
- Any hidden deductibles or aggregate deductible waivers – mentioned as optional; clarify if they apply by default. [Proposed Extraction]

Premium vs coverage

- The proposed plan doubles the sum insured and removes several caps (room-rent, ICU, disease sub-limits) while also eliminating co-payment and extending pre-/post-hospitalisation days.
- However, the premium figures are not provided, so you cannot tell whether the higher coverage translates into a proportionate premium increase. [Comparison Table]

Switching risks

- Waiting periods – pre-existing disease waiting remains 36 months; specific disease waiting improves to 24 months, but the same 30-day initial waiting stays. [Current Extraction] / [Proposed Extraction]
- Continuity of benefits – any no-claim bonus accumulated under the current policy may not automatically transfer; verify portability of bonus. [Current Extraction] / [Proposed Extraction]

- Portability & moratorium – both policies mention portability and moratorium, but the exact conditions (e.g., required continuous coverage) should be confirmed. [Current Extraction] / [Proposed Extraction]
- Potential loss of existing optional riders – if you have any add-ons on the current plan, they may need to be re-purchased under the new policy.
- Please confirm these points with the insurer or a licensed advisor before deciding.*

Questions to ask the insurance advisor / insurer

1. What is the exact annual premium amount for the proposed plan, since it is not stated in the document?
2. Can you confirm the pre-existing disease waiting period is 36 months and explain if any conditions can reduce it?
3. What are the specific diseases listed under the 24-month specified-disease waiting period, and are there any exceptions?
4. How many exclusions are there in total, and can you provide a clear list of all 28 exclusions mentioned?
5. In clause p.43, the company can modify the cashless network at its discretion – what process is followed to inform policyholders of such changes?
6. For the medical examination clause on p.44, who appoints the medical practitioner and what costs are covered by the insurer?
7. Regarding the operating clause on p.2, what exact circumstances trigger cashless hospitalization and are there any limits on claim amounts?
8. Under the premium instalment clause on p.36, are there any additional charges or interest for paying quarterly or monthly?
9. Clause p.17 mentions a single claim limit per policy year – what is the combined maximum amount of base sum insured plus any cumulative bonus?
10. Can you clarify the documentation required for accident cases, such as alcohol consumption history and death summary as noted on p.44?
11. Is there an optional add-on or rider available for newborn coverage, given it is unclear in the current document?
12. Does the policy offer a restoration or recharge benefit after a claim, and if so, what are the terms and limits?

Responsible AI note and limitations

- CoverWise AI is a policy-understanding and decision-support assistant. It does NOT provide financial, legal, medical or insurance advice and does not recommend buying, switching or cancelling any policy. AI extraction can be incomplete or wrong. Always verify every point with the official policy wording, the insurer, or a licensed insurance advisor before making a decision.
- Values were extracted automatically from the uploaded document(s). Page references are given so you can check each value in the original wording.
- Fields marked 'Not clearly found' were not stated clearly; typical values were assumed only for scoring and are listed as 'Verify' flags.
- The Coverage Clarity Score is trained on a synthetic dataset and describes how clear / restrictive the wording is - it is not a measure of claim settlement or insurer quality.
- Personal details (names, phone numbers, emails, PAN, Aadhaar, policy numbers) are masked before any text is sent to the language model.